

# Funding research on women's health



**Women's health research has long been underfunded, in part owing to stigmas associated with conditions that primarily affect women. Equitable health research funding requires transparency from funding agencies, investment in women-centred innovations, support for women in science and a cultural shift in how health issues are viewed.**

Research related to women's health is critically underfunded. In 2020, only 5% of global research and development (R&D) funding was allocated to women's health research. This was split into 4% for women's cancers and 1% for all other women-specific health conditions, with 25% of that further limited to fertility research<sup>1</sup>. Although fertility is an important aspect of healthcare, this narrow focus overlooks a broader spectrum of health conditions that disproportionately affect women, including autoimmune diseases and depressive disorders, as well as conditions that present different symptoms in women than men, such as cardiovascular disease<sup>2</sup>. Therefore, more comprehensive investment is needed across key areas of healthcare, including conditions specific to women, diseases with sex-based differences, conditions with higher prevalence in women and conditions that are historically under-researched in women and female models – this will require diverse and global mechanisms.

In particular, government grants have a vital role in supporting scientific research globally; however, fund allocation is often not representative of disease burden. For example, in the National Institutes of Health, conditions that predominantly affect men receive disproportionately high funding compared with their disease burden<sup>3</sup>. Furthermore, the ability of government agencies to address gaps in health research is hindered by a lack of transparency in funding distribution<sup>4</sup>. Thus, commitments to analyse and publish data on grants for women's healthcare research are crucial.

Philanthropic organizations and global partnerships complement government funding by identifying and addressing critical priorities in healthcare. In this issue, [Ru-fong Joanne Cheng and Mark A. Barone](#) from the Gates Foundation's [Women's Health Innovations](#) team outline how they aim to close healthcare funding gaps by increasing R&D funding for women's health and by shaping policies that prioritize underfunded areas. Similarly, the [Global Alliance for Women's Health](#) provides valuable frameworks to guide investments and engage with experts in the women's health space. This type of public and private sector involvement is vital for supporting women-centred products and technologies.

**“Viewing women's health as a comprehensive and essential part of healthcare will drive better policies, more research and greater investment to benefit society as a whole”**

Investing in scientists can also advance healthcare equity. All-female research teams are 35% more likely to develop medical treatments which serve women's needs, compared to all-male research teams<sup>5</sup>. Organizations such as the [African-German Network of Excellence in Science](#) and the Bayer Foundation's [Women Empowerment Award](#) reward women, particularly individuals from low- and middle-income countries, with grants and awards to support healthcare research and entrepreneurship. Importantly, [more women will be required in the boardrooms and decision-making roles at funding agencies](#) to make funding women's health research a priority.

Outside of academia, the FemTech industry is emerging as a key player in biotech and health innovation. [With about 70% of FemTech companies founded or co-founded by women](#), this sector drives the development of healthcare solutions tailored to women's needs. However, these businesses are largely fuelled by venture capital firms, in which male partners dominate, and FemTech companies often face bias and scepticism, with women's health frequently dismissed as being ‘too niche’<sup>6</sup>. In 2021, FemTech companies reached [US\\$2.5 billion in funding globally, representing about 3% of all digital health investment](#). Reproductive health currently dominates this sector but expanding focus into other areas, guided by input from patients and healthcare providers, could further drive growth. Encouragingly, in Africa, the share of women-led health tech investments rose from [1.4% to 31% between 2022 and 2023](#). Companies such as Rwanda-founded [Kasha](#), which offers customers discrete access to women's health care products, are leading the way, underscoring the importance of person-centred design in FemTech services.

Stigma has compounded funding disparities, as conditions in which individuals are blamed for their ailments are often underprioritized. For example, endometriosis affects roughly 10% of women assigned female at birth<sup>6</sup> but has long been wrongly stereotyped as stemming from life choices, leading to chronic underfunding despite its disease burden. Similarly, multiple sclerosis predominantly affects women and was historically thought to be caused by ‘hysteria’; today, it remains underfunded relative to its impact<sup>4</sup>. Organizations such as the [World Endometriosis Society](#) and the [MS International Federation](#) address disease-specific funding disparities. Closing the gap between men's and women's health research requires not only funding but also a cultural shift in how health issues are viewed.

Women's health is not a niche industry. A 2024 report from the World Economic Forum in collaboration with the McKinsey Health Institute determined that closing investment gaps in women's healthcare could boost the global economy [\\$1 trillion annually by 2040](#), demonstrating the vast economic potential of equitable healthcare funding.

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